



Evaluating Veterans'
Pain Reprocessing Therapy Study

Participant Workbook

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**Pain/Opioid
CORE**



Division of General Internal Medicine
SCHOOL OF MEDICINE
UNIVERSITY OF COLORADO ANSCHUTZ MEDICAL CAMPUS



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Week 1

Summary

All pain is real, and all pain is created in the brain. Pain is an alarm bell activated by the brain to warn us that we are in danger. The brain can make mistakes and activate pain when there is no physical danger.

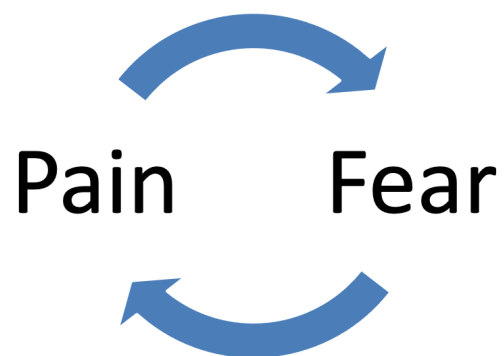
This is called **Neuroplastic Pain**.

- Neuroplastic Pain = Pain that is activated by the brain due to a mistaken perception of threat. It is called ‘neuroplastic’ pain because it is caused by changes (neuroplasticity) in brain processes.
- Experts believe that the majority of chronic pain is neuroplastic pain.¹ For example, 90% of chronic back pain is neuroplastic.²

The perception of threat is the fuel behind neuroplastic pain. When the brain experiences pain as a threat, it is more likely to make a mistake and activate pain. The perception of threat includes fear, frustration, despair, stress, anger, anxiety, annoyance and more—any response that indicates the pain is seen as a threat.

Neuroplastic pain can become chronic because of the **Pain-Fear Cycle**:

1. Pain triggers the perception of threat & feelings of fear or stress.
2. This fear puts the brain on high alert, which causes more pain...
3. Which leads to more fear...
4. Which leads to more pain.



¹ <https://pubmed.ncbi.nlm.nih.gov/34062144/>

² <https://pubmed.ncbi.nlm.nih.gov/27745712/>



To break the cycle of chronic pain, there are 2 tasks in Pain Reprocessing Therapy:

1. Decrease the brain's perception that pain is a threat (including fear, anxiety, anger and hopelessness).
2. Decrease the brain's perception of threat from the non-pain aspects of our lives (stress, pressure, self-criticism).

The objective of Pain Reprocessing Therapy is to teach your brain that you are **safe**.

From the Foreword to **The Way Out**:

“I’ve come to believe that the right ideas about chronic pain, put into practice through mind-body treatment, can have dramatic benefits for many or even most people—even after real injuries that give rise to real pain...”

“Information is a large part of the cure. New information can change our beliefs about the causes of pain and the narratives we tell ourselves about it. Often this change can take work and practice, but it can also happen suddenly, in a flash of insight...”

“You don’t have to have faith that chronic pain can be cured; you just have to believe enough to start working with the ideas in this book with an open mind. Try it and see what happens.”

Tor Wager, PhD
Distinguished Professor of Neuroscience
Dartmouth College



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Practice Check Point 1.1:

Once this week: Watch a video on neuroplastic pain from Dr. Howard Schubiner:

- Breakthrough with Chronic Pain | Howard Schubiner | Talks At Google (53 min)
- <https://www.youtube.com/watch?v=0VyH1laOd2M&t=2809s>

Practice completed?	Day/time	Observations (optional)
✓	Friday, 2pm	Interesting video, lots of helpful information but still many questions

After the video, write down:

- On a scale from 0-10, to what extent do you believe your pain is neuroplastic? (with 0=Not at all and 10=Completely)
- What would help you achieve 100% belief that your pain is neuroplastic? What questions do you have?



Week 1 Resources for Further Learning

LISTENING:

- [Podcast: The Key to Breaking The Pain Cycle](#) (23 min)
 - **URL:** <https://podcasts.apple.com/us/podcast/whats-the-key-to-breaking-the-pain-cycle/id1503847664?i=1000471551067&l=es-MX>
 - **Summary:** Two patients find ways to overcome their fear that their pain comes from a broken body.

SCAN ME



READING:

- The Way Out:
 - Foreword by Tor Wager (pg. ix-xiii)
 - Chapter 1: This Kid's Brain Could Change the World (pg. 1-20)
 - Chapter 2: Pain Is a Danger Signal (pg. 21-31)
 - Chapter 3: Nothing to Fear but Fear Itself (pg. 33-47)
- [Web Article: How the Brain Causes Chronic Pain](#)
 - **URL:** <https://www.healthcentral.com/pain-management/how-the-brain-causes-chronic-pain>



Week 1: My Notes

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Week 2

Summary

Three Barriers to Believing your Pain is Neuroplastic:

1. **Biology:** We are biologically wired to believe that pain is a sign that our bodies are damaged.
2. **Conditioned Responses:** Conditioned responses occur when your brain creates an association between a certain position or activity with the onset of pain. It's easy to believe that this is coming from damage in your body. For example, you get pain every time you sit so you mistakenly believe that sitting hurts your spine. Or you get pain every time you bend over so you mistakenly believe that bending is putting pressure on a bulging disc. But actually, the pain is a **learned response** to a situation that has been painful in the past, even though it is no longer dangerous in the present moment.
3. **Medical Diagnosis:** Another medical professional has told you that your pain is coming from structural damage in your body.

From the **The Way Out**, page 163:

“Because all pain feels like it’s coming from the body, it can be difficult to distinguish between pain that’s physically caused and pain that’s neuroplastic. Luckily, there are certain signs you can look for that point to neuroplastic pain.”



Evidence List: Signs that Your Pain is Neuroplastic

	Personal Evidence
Your pain developed during or after a time of stress.	
Your pain originated without injury, or your injury should have healed by now. <i>Example:</i> Most injuries fully heal after 3-6 months)	
Your symptoms are inconsistent. <i>Example:</i> Your pain is sometimes there or sometimes not or your pain varies in intensity	
You have more than one chronic symptom. <i>Example:</i> You have knee pain, back pain, and headaches.	
Your symptoms spread or move. <i>Example:</i> Your pain moves around your body. One moment it's in your back, the next it's in your ribs.	
Your symptoms are triggered by stress.	



You have pain triggers that have nothing to do with your body. <i>Example:</i> Your pain fluctuates due to weather, sounds, smells, time of day etc.	
You have symmetrical symptoms. <i>Example:</i> You have pain in both wrists, both ankles, both thumbs etc.	
You have delayed pain. <i>Example:</i> You go for a hike and have no pain but an hour later your back begins to hurt.	
You've had childhood adversity.	
You have common personality traits to others with neuroplastic pain. <i>Example:</i> You are prone to self-criticism, putting pressure on yourself, and worrying. Other traits include perfectionism and people pleasing.	
You do not have a clear physical diagnosis for your pain.	



Practice Checkpoint 2.1:

Once a day this week: when you notice your pain, practice telling yourself:
“My body isn’t injured. My brain just thinks I’m in danger.”

	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Friday, 2pm	Felt tense and skeptical
1.			
2.			
3.			
4.			
5.			
6.			
7.			



Practice Checkpoint 2.2:

Once this week: Review your **Evidence List**.

When you finish reviewing, write down your response to the following questions:

On a scale from 0-10, write down to what extent do you believe your pain is neuroplastic (with 0=Not at all and 10=Completely)?

Practice completed?	Day/time	0-10 rating	Observations (optional)



Week 2 Resources for Further Learning

LISTENING:

- [Podcast: How Can I Tell If My Brain is Causing My Pain?](https://podcasts.apple.com/us/podcast/how-can-i-tell-if-my-brain-is-causing-my-pain/id1503847664?i=1000475895716) (30 min)
 - **URL:**
<https://podcasts.apple.com/us/podcast/how-can-i-tell-if-my-brain-is-causing-my-pain/id1503847664?i=1000475895716>
 - **Summary:** Two patients determine whether their pain is caused by damage in their bodies or caused by their brain.

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READING:

- The Way Out
 - Chapter 4: Embracing a New Perspective (pg. 49-64)
 - Appendix (pg.163-169)

Week 2: My Notes



Week 3

Summary

Negative thoughts can keep you in a state of high alert and release stress hormones which makes it more likely that you'll experience chronic pain. Negative thoughts can be about pain (e.g., "I will be in pain the rest of my life") or about life outside of pain. Common negative thoughts outside of pain include pressure, worry, and criticism.

Steps for dealing with negative thoughts:

1. Notice the negative thought.
2. Don't buy into the thought (Remind yourself these thoughts are just our brain's way of keeping us in a state of anxiety and high alert; they aren't necessarily true)
3. Replace the negative thought with a message of safety, comfort or reassurance. Or, imagine a good friend is having the same negative thoughts as you are. What would you tell them?

Gratitude can decrease stress hormones like cortisol, which sensitize your nervous system and cause pain. Gratitude can increase calming hormones, like oxytocin and serotonin.

Gratitude Meditation

1. Think of 5 things you are grateful for (These can be small things.)
2. After you think of the fifth, stop thinking and bring your awareness down to the physical sensation of your heart. Notice the softening in your chest and sensation of gratitude in your heart. Take a few moments to feel this good sensation before returning to your life.

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Practice Checkpoint 3.1

Once a day: Catch a negative thought (from your “Inner Drill Sergeant”) and write them down. Imagine a good friend is having these thoughts. What advice would you give your friend? Next to the written negative thought, write a message of safety for yourself.

	Practice completed?	Day/time	Negative Thought	Message of Safety
Ex.	✓	wed, 12pm	I will always have back pain.	My body isn't injured. My brain just thinks I'm in danger.
1.				
2.				
3.				
4.				
5.				
6.				
7.				



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Practice Checkpoint 3.2

Once a day: Listen to the Gratitude Meditation, ideally first thing in the morning.

	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Saturday, 6pm	Felt warmth in my chest. Felt a little lighter.
1.			
2.			
3.			
4.			
5.			
6.			
7.			



Week 3 Resources for Further Learning

LISTENING:

- Podcast: Can Changing my Thoughts Improve my Pain? (30 min):
 - **URL:** <https://podcasts.apple.com/us/podcast/can-changing-my-thoughts-improve-my-pain/id1503847664?i=1000491457739>
 - **Summary:** A patient learns how to decrease her negative thoughts and put less pressure on herself.



READING:

- The Way Out, Chapter 8: Getting Good at Feeling Good (pg.121-138)
- TMS Wiki: Pressure and Criticism
 - **URL:** <https://www.tmswiki.org/forum/threads/day-7-pressure-and-criticism.16512/>
 - **Summary:** Fear, pressure, and criticism activate our danger signals, prevent our brains from feeling safe, and perpetuate a cycle of pain. It's important to recognize these behaviors and the impact they can have, so that we're able to change them.



Week 3: My Notes



Week 4

Summary

- **Stress** causes your brain to interpret everything through a lens of danger. This creates more pain.
- **Trauma** can make a brain more prone to being on the lookout for danger and making someone feel constantly “vigilant” or “on guard.”

A **safety boost** helps teach the brain to become less vigilant over time.

Three Steps of a Safety Boost:

1. Put your hand on your chest.
2. Give yourself a message of safety by saying something to yourself like “right now, I am safe”, or, “this is really not as bad as I am making it out to be”.
3. Allow your exhales to slow and lengthen.

Habits and behaviors, like constant phone or TV use, can keep the brain in a high alert, stressed state, which leads to more pain. We gravitate toward stressful activities because they feel familiar to our brains.

Leaning into positive sensations teaches our brain to come out of a high alert state.

Two Steps for Leaning into Positive Sensations

1. Find a sensation that feels good (Examples: the sun on your skin, the breeze on your face, the feeling of your breath rising and falling).
2. Rest a part of your awareness on this sensation. See if you can enjoy it! If your mind wanders, try and keep a part of your awareness of the pleasant sensation even as your mind wanders.

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Practice Checkpoint 4.1

Once a day this week: Give yourself a safety boost when you are feeling tense or on guard. Write down what safety message you gave yourself.

	Practice completed?	Day/time	Message of Safety
Ex.	✓	Tuesday, 4pm	In this moment right now, I am safe.
1.			
2.			
3.			
4.			
5.			
6.			
7.			

Practice Tip! Visiting a memory of a time you were physically or psychologically wounded and asking that version of yourself what they needed to hear in that moment is the best way to figure out what messages of safety you need in the current moment.



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Practice Checkpoint 4.2

Once a day: Listen to the Leaning into Positive Sensations Meditation. Record what positive sensation you leaned into or any other observations.

	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Sunday, 5pm	Feeling the warmth of the shower after a long day
1.			
2.			
3.			
4.			
5.			
6.			
7.			

Practice Tip! If you are having a hard time finding a positive sensation, either use thoughts of gratitude to create more positive sensations or bring your awareness to something that feels neutral, like the sensation of your breath rising and falling.



Week 4 Resources for Further Learning

LISTENING:

- Podcast-How do I get to the root of my pain? (33 min)
 - **URL:** <https://podcasts.apple.com/gb/podcast/how-do-i-get-to-the-root-of-my-pain/id1503847664?i=1000487933899>
 - **Summary:** A patient learns to break his high alert patterns and feel more relaxed and calm.



READING:

- The Way Out, Chapter 7: Breaking the High-Alert Habit (pg.105-120)

Week 4: My Notes



Week 5

Summary

A **corrective experience** is anytime you interact with something you once feared and come away from the experience understanding that it's less dangerous than you thought. To overcome chronic pain, you want to approach your pain in a way that is going to allow you to experience it in a way that feels safe.

Somatic Tracking teaches your brain that the pain is safe and not to be feared. There are 3 components of somatic tracking:

1. **Mindfulness:** Pay attention to your pain with curiosity. Watch your pain like watching clouds passing overhead or as if you are watching a movie (maybe an action movie on days when symptoms are more intense).
2. **Safety Reappraisal:** Giving yourself messages of safety while you feel your pain. "There's nothing wrong with my muscles, my body is healthy, my brain is making a misinterpretation."
3. **Positive Affect Induction** (Lightening the mood!): Loosen your body physically, let go of tension and see if you can watch your pain with less urgency. Maybe let a smile crack the corners of your lips. Think of a nice memory.

Outcome Independence means not being overly concerned about getting rid of your pain when you use a tool like somatic tracking. Your long-term goal can be getting out of pain but your short-term goal with somatic tracking should be to feel safer and more relaxed as you watch your pain. Your brain will take care of your long-term goal.

From the **The Way Out**, page 66:

"By being outcome independent, you reinforce to your brain that the pain is safe. Eventually, your brain will learn this lesson and your pain will fade."



Practice Checkpoint 5.1:

Once a day this week: When your pain is at a moderate or low level, listen to the Somatic Tracking Meditation and follow the instructions. Don't worry about learning the steps. See if you can just follow along without pressure. After the recording has finished, review the 3 components of somatic tracking above.



	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Monday, 10am	Felt the pain change in intensity
1.			
2.			
3.			
4.			
5.			
6.			
7.			

What aspects of somatic tracking give you the most difficulty?



Week 5 Resources for Further Learning

LISTENING

- Podcast: How Can Somatic Tracking Help My Pain? (33 min):
 - **URL:** <https://podcasts.apple.com/us/podcast/what-is-somatic-tracking-and-how-can-it-help-my-pain/id1503847664?i=1000478323015>
 - **Summary:** 3 patients are led through somatic tracking and find ways to alleviate their fear and symptoms.



READING:

- The Way Out, Chapter 5: Somatic Tracking (pg.65-78)
- TMS Wiki- Pain Reprocessing (Somatic tracking with a patient)
 - **URL:** <https://www.tmswiki.org/forum/threads/day-11-pain-reprocessing.16616/>
 - **Summary:** Sample clip of somatic tracking with a patient with headache pain
- TMS Wiki-Somatic Tracking Big Picture
 - **URL:** <https://www.tmswiki.org/forum/threads/day-9-somatic-tracking.16532/>
 - **Summary:** How to do somatic tracking



Week 5: My Notes



Week 6

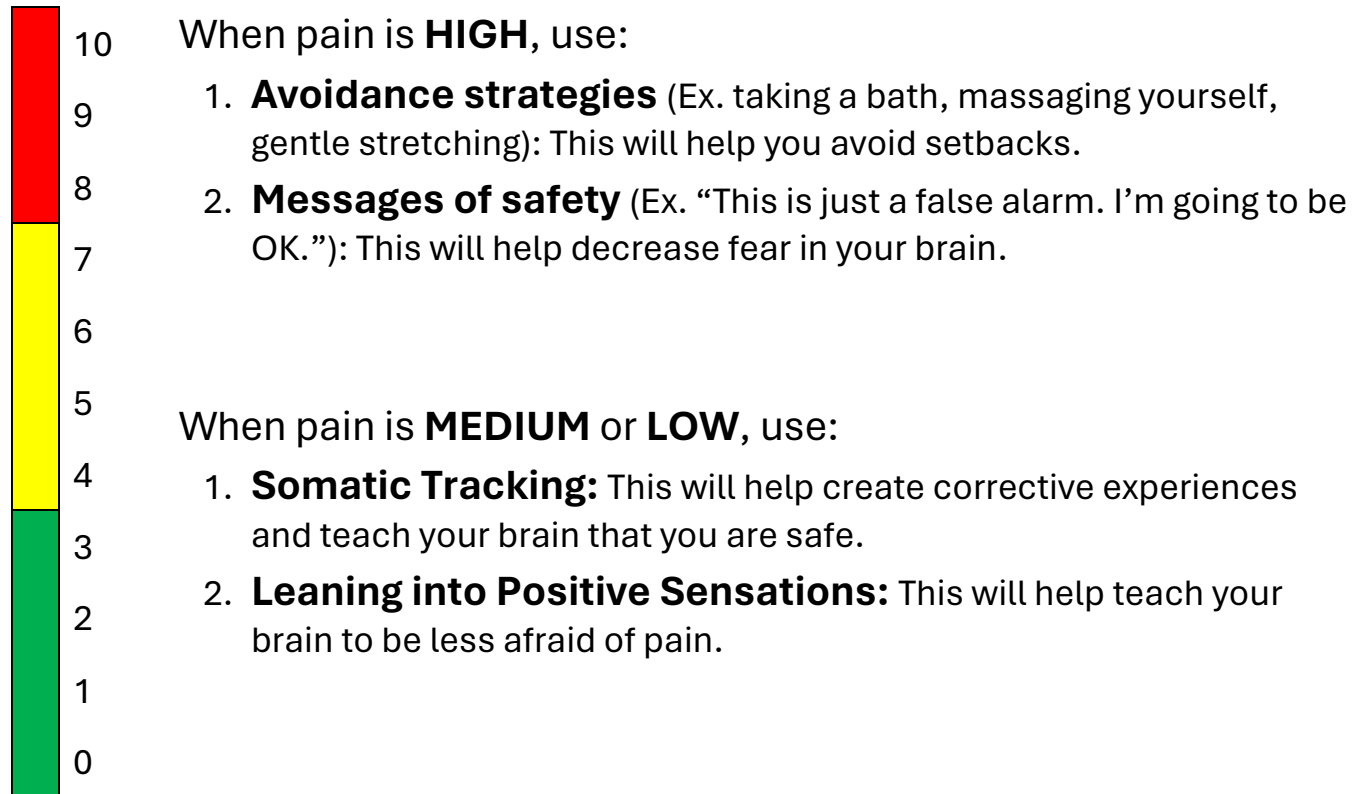
Summary

Somatic tracking is much easier when you **calm down your system** first.

Physically calming your system can include slowing your breathing, extending your exhales longer than your inhales, scanning your body and letting go of tension/bracing.

Psychologically calming your system can include visualizing a time or a place you felt at play, picturing people in your life who you care about, thinking of five things you are grateful for.

Trust The Process (or when to use Somatic Tracking):





Practice Checkpoint 6.1:

Once a day this week: For 3 minutes, practice slowing your breath down and extending your exhales to be longer than your inhales. When your slower breathing feels comfortable, scan your body from the top of your head and move downwards. Be on the lookout for tension. See if you can let go of any bracing you are doing. Lastly, put a smile on your face. It doesn't have to be a big smile, just a small one to signal to your brain that you are safe.

Next, practice using somatic tracking for a few minutes on your pain. Does it feel easier to do now that you are in a calmer space? Do you feel less fear surrounding your pain?

	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Friday, 9pm	Challenging at first, then started to get the hang of it
1.			
2.			
3.			
4.			
5.			
6.			
7.			



Bonus Practice Checkpoint 6.2 (Optional)

Twice this week: As a bonus practice, try to listen to the Gratitude Somatic Tracking Meditation (or you can try it on your own). Think of 5 things you are grateful for and feel the sensation of gratitude in your chest. Now move your attention to somatic tracking your pain. Notice how it feels easier to use somatic tracking when you practice gratitude first.

SCAN ME



	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Saturday evening	Slightly easier after the gratitude exercise
1.			
2.			



Week 6 Resources for Further Learning

READING:

- The Way Out, Chapter 6: The Process (pg. 79-104)

Week 6: My Notes



Week 7

Summary

Conditioned Response: A position or activity that triggers pain (ex. standing up, bending over, sitting for too long).

Remember: It's not the position or activity you are doing that physically causes the pain. Your brain has learned that this specific situation is dangerous and turns the pain on to warn you of the danger.

Goal: Teach your brain that these positions and activities are safe again

Tool: Somatic tracking of your pain in positions or activities that cause you pain

If somatic tracking for your pain feels too difficult, practice using somatic tracking on other fear sensations in your body first. These sensations include:

- Muscle tension
- Anxiety feelings of tightness in your chest, throat or stomach.
- Faster heartbeat
- Shorter breath
- Temperature fluctuations like waves of heat or even cool numbness
- Tingling feeling in skin

When these sensations calm down, using somatic tracking on your pain will be much easier.

From the **The Way Out**, page 55:

“Conditioned responses can make it hard to believe that your pain is caused by your brain...Your brain has developed an association between a position or activity and the onset of pain. But just as these associations can be learned, they can be unlearned.”



Practice Checkpoint 7.1

Once a day: Experiment with using Somatic Tracking on the anxiety or fear sensations in your body. See if you can ignore your pain and instead bring your awareness to the other sensations of fear like tightness in your chest or tingling in your skin.

	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Thursday, 12pm	Warmth in the chest and elevated heart rate
1.			
2.			
3.			
4.			
5.			
6.			
7.			



Practice Checkpoint 7.2

Leaning into Positive Sensations Review

Remember, slowing down and bringing your attention to positive sensations can help your brain relax and teach your brain to view your bodily sensations through a lens of safety (ex. Warm water in a hot shower, sun on your face, stretching muscle).

If you are having difficulty feeling positive sensations, start focusing on neutral sensations (ex. Sensation of your breath, rubbing your hands together to create warmth).

Times to use leaning into positive sensations:

- When you're worrying
- When you are constantly checking or focused on pain
- When you are out of pain and want to feel more relaxed

Once this week: Listen to the Leaning into Positive Sensations recording. Later in the week, while you are doing a mindless task this week (folding laundry, cooking food, cleaning etc.) See if you can lean into a positive sensation while you do the task for at least 2 minutes.

SCAN ME



	Practice completed?	Day/time	Observations (optional)
Ex.	✓	Tuesday morning	Leaned into the smell of coffee while making breakfast
1.			



Week 7 Resources for Further Learning

READING:

- TMS Wiki- Conditioned Responses
 - **URL:** <https://www.tmswiki.org/forum/threads/day-18-conditioned-responses.16609/>
- TMS Wiki- Somatic Tracking with Anxiety
 - **URL:** <https://www.tmswiki.org/forum/threads/day-10-somatic-tracking-ii-anxiety-strikes-back.16540/>



WATCHING:

- YouTube- Greg Overcomes his conditioned response of sitting
- YouTube Clip- Felicia overcomes her fear/pain of turning her neck



Week 7: My Notes



Week 8

Summary

The ***main objective of Pain Reprocessing Therapy*** is to teach your brain that you are safe.

Common Problems and Solutions

Problem	Solution / Tool
Belief that you are structurally damaged	• Review & update your Evidence Sheet • Listen to recovery stories (Podcast: Tell Me About Your Pain)
Severe Pain	• Use avoidance strategies • Send messages of safety
Mild to Moderate Pain	• Use Somatic Tracking • Practice leaning into positive sensations
Constant self-criticism, worry, or pressure thoughts	1. Catch the thoughts 2. Don't buy into them 3. Replace with messages of safety
Constant state of high alert	• Safety boost • Decrease stressful habits (ex: phone usage) • Listen to Positive Sensations & Gratitude meditations



Three Stages of Relapse/Extinction Burst

Relapses or extinction bursts occur because your brain is trying to return to an old pattern that is familiar. They can occur out of the blue or have an obvious trigger like an increase of anxiety or encountering a triggering situation (family gathering, work outing, etc.)

1. **Panic:** In this stage, your fear is high. Use messages of safety and avoidance strategies until the panic passes.
2. **Forcing It:** In this stage, you try to force the tools to work to make the pain go away. Try to remind yourself of why the tools worked in the first place. See if you can make your brain feel safer and create a feeling of lightness and relaxation rather than desperation when you use the tools.
3. **Back to Basics:** In this stage, you remember that the tools are meant to make your brain feel safe rather than turning your pain off. You stop worrying about the pain on a moment-to-moment basis.

From the **The Way Out**, page 146:

“Resilience is helpful for every part of the pain recovery process: relapse, extinction bursts, and especially setbacks... As bad as setbacks feel, they actually give us opportunities to build resilience. And building resilience is what leads to a reduction of fear.”



Practice Checkpoint 8.1

Once this week: Review your **Evidence List** from Week 2.

When you finish reviewing, write down your response to the following questions:

On a scale from 0-10, write down to what extent do you believe your pain is neuroplastic (with 0=Not at all and 10=Completely)?

Practice completed?	Day/time	0-10 rating	Observations (optional)

Finally, consider writing out an updated Evidence List using the blank template on page 39-40.

Has your evidence changed since Week 2? What are the important changes?
Is there anything surprising in your updated Evidence List?



Bonus Practice Checkpoint 8.2 (Optional):

Continue to experiment with the tools and find the combination that works best for you. Remind yourself that recovery is not a straight line. Take time to reflect on your experience over the past 8 weeks. Everyone has a “pain story”— the story of their experience with chronic pain and its meaning in their lives. What is your story? How has your pain story changed over the past 8 weeks?

As a bonus practice, write down your pain story using the 3 questions below as a guide. If you need more space, you can find another pain story template on page 44.

1. What was it like?

2. What happened?

3. What is it like now?



Week 8 Resources for Further Learning

READING:

- The Way Out, Chapter 9: Relapses, Resilience, and Recovery (pg. 139-154)

- TMS Wiki- Extinction Bursts

- **URL:** <https://www.tmswiki.org/forum/threads/day-19-extinction-bursts.16651/>



- TMS Wiki- Paving Your Own Path

- **URL:** <https://www.tmswiki.org/forum/threads/day-21-paving-your-own-path.16670/>



Week 8: My Notes



Appendices

Appendix 1. Links to Guided Meditations

Appendix 2. Weekly Survey Tracking

Appendix 3. Extra Evidence List Table

Appendix 4. When To Use Somatic Tracking?

Appendix 5. Threat Assessment

Appendix 6. Writing Your Pain Story

Appendix 7. Notes



Appendix 1. Links to Guided Meditations

Week	Guided Meditation	QR Link
Week 3	<u>Guided Gratitude Meditation</u> Time: 3 min. Summary: Practice gratitude to help reduce the brain's perception of threat	
Week 4	<u>Leaning into Positive Sensations</u> Time: 5 min. Summary: Bring your attention to pleasant physical sensations to develop a new skill & new neural pathways	
Week 5	<u>Guided Somatic Tracking</u> Time: 4 min. Summary: Practice the three components of mindfulness, safety reappraisal & positive affect induction	
Week 6	<u>Gratitude & Somatic Tracking</u> Time: 6 min. Summary: In this 2-part guided meditation, you will connect the skills you've learned in prior weeks	



Appendix 2. Weekly Survey Responses

	Pain (0-10)	Interference with Activity (0-10)	Interference with Enjoyment (0-10)	Total
Week 1				
Week 2				
Week 3				
Week 4				
Week 5				
Week 6				
Week 7				
Week 8				
Week 16				



Appendix 3. Extra Evidence List Table

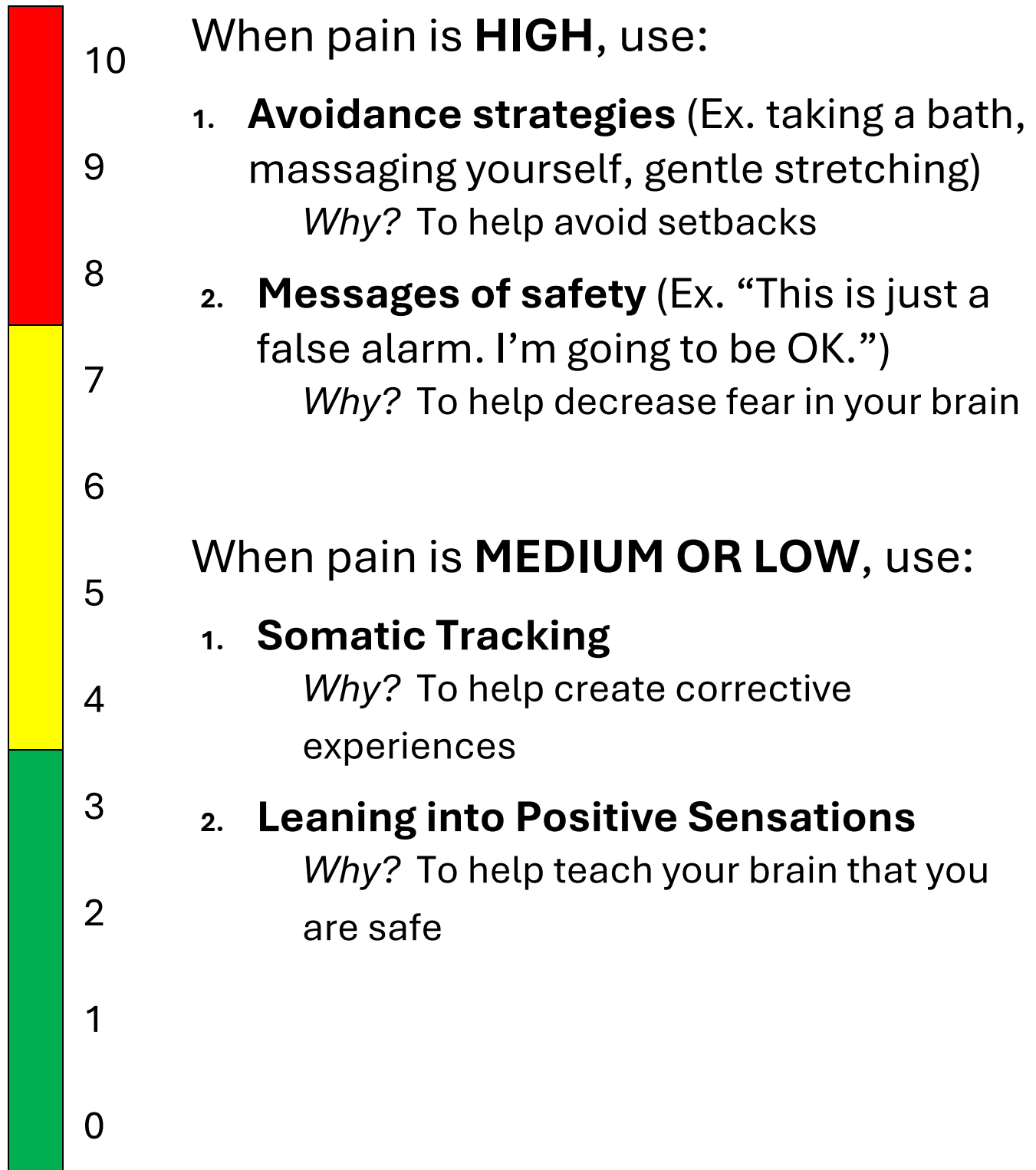
	Personal Evidence
Your pain developed during or after a time of stress.	
Your pain originated without injury, or your injury should have healed by now. <i>Example:</i> Most injuries fully heal after 3-6 months)	
Your symptoms are inconsistent. <i>Example:</i> Your pain is sometimes there or sometimes not or your pain varies in intensity	
You have more than one chronic symptom. <i>Example:</i> You have knee pain, back pain, and headaches.	
Your symptoms spread or move. <i>Example:</i> Your pain moves around your body. One moment it's in your back, the next it's in your ribs.	
Your symptoms are triggered by stress.	



You have pain triggers that have nothing to do with your body. <i>Example:</i> Your pain fluctuates due to weather, sounds, smells, time of day etc.	
You have symmetrical symptoms. <i>Example:</i> You have pain in both wrists, both ankles, both thumbs etc.	
You have delayed pain. <i>Example:</i> You go for a hike and have no pain but an hour later your back begins to hurt.	
You've had childhood adversity.	
You have common personality traits to others with neuroplastic pain. <i>Example:</i> You are prone to self-criticism, putting pressure on yourself, and worrying. Other traits include perfectionism and people pleasing.	
You do not have a clear physical diagnosis for your pain.	



Appendix 4. When To Use Somatic Tracking?





Appendix 5. Threat Assessment

Area	Resources	Threats
<u>Power of the Mind</u>		
<u>Moving the Body / Physical Activity</u>		
<u>Surroundings</u>		
<u>Personal Development</u>		
<u>Food & Drink / Nutrition</u>		
<u>Recharge / Sleep</u>		
<u>Family, Friends & Co-Workers</u>		
<u>Spirit & Soul</u>		

* These 8 areas have been adapted from the VA's Circle of Health.

For more information, click the links above or check out:

<https://www.va.gov/WHOLEHEALTH/circle-of-health/index.asp>





Appendix 6. Writing Your Pain Story

1. What was it like?

2. What happened?

3. What is it like now?



Appendix 7. Notes

Thank you for participating in the

ESPRiT Study

With any study-related questions,
please email VHAECHPRT@va.gov

SCAN ME



VA Eastern Colorado Healthcare System
Chronic Pain & Wellness Center
Veterans Crisis Line

303-399-8020
720-723-3113
Dial 988 or
Text 838255

